June 13, 2025

The Honorable Mehmet Oz, M.D.
Administrator
Centers for Medicare & Medicaid Services
U.S. Department of Health and Human Services
Hubert H. Humphrey Building
200 Independence Avenue SW
Washington, DC 20201

Submitted electronically at https://www.regulations.gov

Re: Request for Information; Health Technology Ecosystem (RIN: 0938-AV68)

Dear Administrator Oz:

Premier Inc. appreciates the opportunity to submit comments to the Centers for Medicare and Medicaid Services (CMS) and the Assistant Secretary for Technology Policy/Office of the National Coordinator for Health Information Technology (ASTP/ONC) on opportunities to improve data interoperability and the broader health technology infrastructure for all stakeholders. Premier supports the Administration's efforts to empower patients through the effective and responsible adoption of technology in healthcare. In our detailed comments below, Premier specifically recommends that CMS focus on the following:

- Harmonizing interoperability regulations to promote greater market competition;
- Updating meaningful use criteria, certification standards for health IT and the overall federally-driven interoperability incentive structure to ensure that data usability for quality and process improvement are adequately incentivized;
- Clearly defining information blocking and implementing and enforcing more stringent information blocking penalties on health data ecosystem participants beyond providers and provider organizations;
- Extending interoperable electronic health record incentives to post-acute and continuum of care providers;
- Developing a holistic framework of both financial and non-financial incentives to promote value-based care participation, which in turn results in greater adoption of tech-enabled solutions;
- Mandating a patient-directed data sharing mechanism such as Blue Button 2.0 in certified electronic health record technology (CEHRT) criteria;
- Requiring more standardized data sharing by CMS-regulated health plans to providers and patients; and
- Improving Risk Adjustment Data Validation (RADV) audit processes by piloting a program that relies on CMS-approved, real-time self-audit technology rather than perpetuating a pay-and-chase model.

Our recommendations are discussed in greater detail below.

I. BACKGROUND ON PREMIER INC.

Premier is a leading healthcare improvement company and national supply chain leader, uniting an alliance of 4,350 hospitals and approximately 325,000 continuum of care providers to transform healthcare. With

integrated data and analytics, collaboratives, supply chain solutions, consulting and other services, Premier enables better care and outcomes at a lower cost.

A Malcolm Baldrige National Quality Award recipient, Premier plays a critical role in the rapidly evolving healthcare industry, collaborating with healthcare providers, manufacturers, distributors, government and other entities to co-develop long-term innovations that reinvent and improve the way healthcare is delivered nationwide. Headquartered in Charlotte, North Carolina, Premier is passionate about transforming American healthcare.

II. FOUNDATIONS OF HEALTHCARE'S TECHNOLOGY ECOSYSTEM

Fundamentally, Premier believes that the foundation of a modern, effective healthcare technology ecosystem rests on three interdependent pillars: patient-centered records, digital health technologies, and a robust privacy and cybersecurity framework. Patient-centered records ensure that individuals own and can access their health data, with all medical information linked under a single, longitudinal patient identifier. This structure enables continuity of care and empowers patients to actively participate in their health journey. Digital health software and tools further enhance care delivery, while a strong privacy and cybersecurity infrastructure ensures that these components are interoperable in a secure, trustworthy manner. ***These three pillars must work in concert to put technology to work for good in healthcare, protecting patient data while enabling seamless information exchange across the healthcare system.***

Patient-Centered Records

Patients must be at the core of tech-enabled healthcare. In the push to collect more patient data, improve data sharing and end information blocking, it is crucial to recall that the purpose of collecting and sharing healthcare data is to drive better patient outcomes at lower costs. Healthcare data regulations and incentives that lose sight of patients' best interests miss the mark.

Patients deserve longitudinal and portable health records that incorporate a patient's entire medical history, regardless of where they received care, and follow the patient when they move to a new neighborhood, city or state. The best way to make health records work for patients is tokenization, which anonymously links encrypted patient data across health systems, providers and appointments. Tokenization can help link together longitudinal patient records that are fully portable, helping patients to have a more complete view of their care journey and giving providers the best possible insights into how to care for each patient.

Digital Health

"Digital health" includes software, technologies and systems that engage patients for care delivery, exchange protected health information (PHI) and enable workflows. Premier considers six categories of digital health technologies: administrative and enterprise IT systems, clinical enablement technology, health information management (HIM), imaging systems, patient engagement and consumerism, and virtual and remote care. The way providers deliver and patients engage with care is expanding beyond hospital and clinic walls, incorporating data sources from nutrition apps, ambient monitoring tools and virtual visits. In response, CMS must consider how to best get data from these tools and technologies into the hands of the appropriate end user – be that the patient or provider – to incentivize larger-scale adoption.

As CMS considers the most appropriate channel through which to incentivize and distribute digital health technologies, it should commit to identifying which technologies need to be integrated with or distributed through hospitals and health systems to truly drive value and appropriately collect and utilize new data sources. As the largest payer in the US, CMS can leverage its authorities to drive adoption and implementation in a provider setting directly – and as some of the largest nodes in the health tech ecosystem, hospitals and health systems have the volume of patient visits, data assets and infrastructure to help bring innovation to scale quickly.

Privacy and Cybersecurity Framework

One of the greatest barriers to the large-scale diffusion of innovative digital health applications is the lack of a single federal privacy law. A comprehensive, national privacy framework is necessary to provide a reasonable level of consumer confidence that businesses will protect their data, sensitive or otherwise. The current patchwork, state-driven approach to privacy policy has resulted in inconsistent data privacy practices that have resulted in patient's distrusting the system. Patients must be able to trust the processes for storage, handling and use of their data, particularly as patient data is increasingly used to train AI algorithms – missing data from patients wary of data sharing could add to data siloing and perpetuate AI algorithmic biases, and it is incumbent on CMS, ASTP/ONC and other health data stakeholders to proactively educate patients on these tradeoffs.

Additionally, healthcare privacy enforcement authorities are currently diffused across a wide swath of the federal government. The Health Insurance Portability and Accountability Act (HIPAA) only delegates privacy enforcement authority to the Department of Health and Human Services (HHS) Office of Civil Rights (OCR) for a narrow subset of covered entities. With the proliferation of a healthcare data ecosystem outside of the traditional payer-provider exchange, enforcement authority falls to the Federal Trade Commission, the Department of Justice and other agencies for a broad range of privacy- and data-related violations. There is significant, urgent need for a unified baseline set of enforcement priorities to create a harmonized enforcement authority for the portion of healthcare ecosystem outside of the bounds of HIPAA.

Furthermore, as CMS builds the digital pipelines that will carry the health data of every American citizen, cybersecurity must be front of mind. America cannot afford to expose bulk health, financial and biometric data to adversaries and bad actors. How the US regulates digital health and AI - and the enabling technologies that power it - will transform healthcare. However, making America's healthcare system the most attractive in the world for innovators and visionaries also makes our digital health infrastructure an attractive target for cyber criminals.

America has learned a difficult lesson about the threat of becoming reliant on untrustworthy technology. From telecommunications infrastructure to solar power inverters to port cranes, much of this country's critical infrastructure has faced the threat that unsecured software and hardware poses to essential functions. A fresh focus on security must begin with trustworthy digital infrastructure. Early and sustained leadership and innovation is essential to provide America's critical infrastructure, especially healthcare, with the reliable, trustworthy tools that it needs.

III. RATE-LIMITING FACTORS IN THE CURRENT TECHNOLOGY ECOSYSTEM

To appropriately unlock the value of healthcare data and technologies to improve patient outcomes and decrease costs, CMS and ASTP/ONC must understand the barriers and opportunities facing patients, providers, payers and other nodes in the current health tech ecosystem.

Interoperability

Premier believes that the status quo for data sharing in healthcare does not keep pace with new use cases and fails to deliver on the promise of artificial intelligence (AI), which stands to revolutionize the practice of medicine once data sharing obstacles are overcome. The past two decades of federal policy built a health information technology ecosystem that does not efficiently support the seamless sharing of data and high-quality care.

Interoperability is key to the promise of a modern US healthcare system. Americans have access to more data about their health, wellness and medical conditions than at any time in history – data generated in hospitals and at home, by providers and by consumer applications. Over the past 15 years, CMS and ASTP/ONC policies evolved from Meaningful Use and the promise of electronic health records (EHRs) to

enable simple data exchange, to FHIR APIs and the recognition that interoperable data elements can drive value through longitudinal records and real-world evidence-informed analytics. However, the demonstrated value of healthcare data has created incentives for certain players to restrict access as much as possible – making a small subset of health IT vendors wealthy while draining value from the overall healthcare ecosystem.

America should be entering the heyday of digital health, allowing traceable, trackable and easily transferable data that powers AI and more effective care, but regulations dating back to the HITECH Act and meaningful use have diluted market incentives for innovation and digital adoption. ***Premier urges CMS to examine the existing regulatory frameworks – including the HITECH Final Rule, HTI-1, and all subsequent HTI rulemaking – that saddle technology-enabled healthcare with burdensome and arbitrary qualifying thresholds, and instead introduce new market-aligned definitions of meaningful use of health IT, interoperability, and information blocking.*** It is clear to healthcare providers and patients that today's system is not interoperable, and any statutory definition that says otherwise is untethered from the reality of healthcare delivery.

Interoperability must also be extended to post-acute and continuum of care settings*.* When the HITECH Act offered incentives to increase EHR adoption across acute care providers, it initiated a new "baseline" for interoperability. However, providers that were not eligible for HITECH incentives now lag, often due to an inability to invest the upfront cost necessary to implement a major EHR system. This has led to large bodies of patient data being "disconnected" from the larger healthcare data ecosystem. To generate the savings and benefits of remote and virtual digital health technologies across healthcare, interoperability must be achievable for all providers - including providers that have not yet implemented major EHRs.

Lastly, public health data must be better incorporated with healthcare data from providers to create a robust infrastructure in the best interest of patients. Public health interoperability initiatives offer valuable lessons about data standardization and sharing. For example, CDC's Data Modernization Initiative recognized the workforce and digital infrastructure needed to underpin interoperability, and several state public health agencies have also passed laws requiring standardized data reporting for certain public health data elements. However, as operational crises during the pandemic demonstrated, there is a significant need for federal and state investment to make public health data fully interoperable with the health data ecosystem, including upgrading outdated hardware and software, expediting data collection and establishing unifying standards across all levels of the public health system.

Data Standardization

Data standardization across the healthcare ecosystem has proven elusive despite extensive regulatory efforts and investment in health technology over the past two decades. Notably, although industry and federal experts continue to push for a standardized, Fast Healthcare Interoperability Resources (FHIR)-based interoperability infrastructure, the limitations of FHIR application programming interfaces (APIs) and the select data elements of USCDI limit the usefulness of this standardization outside of specific use cases.

Standardization in healthcare data is currently limited to a subset of elements. Across multiple categories of data – including quality measures, admission, discharge and transfer (ADT) systems and notifications and lab and clinical data – data is shared among EHR vendors, payers and providers without using standardized elements. Standardization would simplify hospital quality and performance improvement efforts, reducing patient costs and driving better outcomes. It would also put an end to "infeasibility by design," the claim by health IT vendors that they lack the technical ability to incorporate certain types of data sharing or reporting.

Workforce

New technologies, including remote patient monitoring and AI, are transforming the delivery of care. However, even if health systems invest in new technologies, providers themselves must drive adoption.

Among providers, there is a growing acceptance of technology as a workforce extender, particularly when it is seamlessly integrated into clinical workflows, and an increasing share of the healthcare workforce is open to adopting new tools. However, this rising digital nativism is not being strategically cultivated by formal medical education and training programs. As a sector, healthcare must find ways to integrate digital health technologies into educational curriculums at all levels, including professional certifications and continuing education.

Anecdotal evidence indicates that nurses and practitioners believe in the value of virtual care to balance workload and expand access to care, particularly in rural areas. Optimized and evidence-backed models have the potential to improve access to care in rural communities, increase savings and reduce chronic disease costs. In the face of nursing shortages, the existence of a center of excellence for optimized virtual nursing services could provide care to entire regions, offsetting workforce capacity challenges and reducing brain drain.

However, not all health system executives have seen the value of the significant up-front investment necessary to build out virtual care services. This is compounded by the fact that, among organizations that adopt virtual care and virtual nursing programs, each has its own model, its own measure for value and effectiveness and its own unique data metrics to evaluate its model. This dynamic makes it challenging for peer organizations to meaningfully consider their own investments in virtual care. There is currently no strong national data to benchmark virtual care services, and thus no way to evaluate relative quality impact. CMS and ASTP/ONC should evaluate potential policy levers to promote this benchmarking as a priority. Health systems are testing models of care, but without cross-sectional, national longitudinal studies to measure value, it will be impossible to appropriately reimburse these services for the value they provide patients.

Financing

Technology-enabled care holds promise for reducing costs across the healthcare sector while improving patient outcomes. Realizing the full value of these tools requires widespread provider adoption, which in turn requires incentives aligned toward broad provider uptake. The movement to value-based care unlocks financial flexibility for providers, potentially providing a path to stable financing for digital health innovation in clinical settings. However, fee-for-service reimbursement for novel technologies lags, disincentivizing purchasers – particularly providers facing tight financial margins – from investing in innovation. Providers need reliable reimbursement for certain types of remote patient monitoring (RPM) codes and other key components of programs such as CMS-approved hospital at home programs.

Coverage Determinations for Novel Therapies and Innovative Technologies

In Medicare and among payers more broadly, delays between device approval from the Food and Drug Administration (FDA) and coverage determinations for breakthrough medical devices deny patients access to potentially life-saving new technologies. During the previous administration, CMS rescinded a proposed rule authorizing the Medicare Coverage of Innovative Technology (MCIT) pathway, which would have required Medicare to provide interim coverage of medical devices in the FDA's Breakthrough Device program following authorization. This policy was in response to the backlog of devices waiting for Medicare coverage determinations. The previously proposed policy change would have increased patient choice and fostered competition among new and emerging medical technologies. Instead, CMS replaced this pathway with the Transitional Coverage for Emerging Technologies pathway, which drastically increases administrative burden and place additional hurdles between Breakthrough Device-designated innovations and coverage. Additionally, CMS has indicated that the pathway is limited to no more than five breakthrough technologies per year, which artificially suppresses competition. ***Premier urges CMS to ensure that current regulations do not decrease competition for innovative medical technologies.*** When the government picks winners, patients lose.

In addition, real-world data (RWD) and real-world evidence (RWE) are increasingly being utilized to inform regulatory submissions to the FDA and provide actionable insights that improve research and accelerate the process of bringing new innovations to patients. RWE also plays an important role in gaining insights into safety, efficacy, patient outcomes, and cost-effectiveness of drugs and devices. Unfortunately, CMS has yet to realize the full benefits of RWE in making coverage decisions for non-FDA-approved indications thereby delaying and denying access to potentially life-saving, life-sustaining, and life-altering medical interventions. ***Premier urges CMS to incorporate the use of RWE into future coverage determination pathways.***

Integrating the Digital Health Supply Chain

To fully unlock the potential of new digital health technologies to improve outcomes and lower costs, data from these technologies needs to make it into patient medical records. The challenge facing healthcare providers, as well as CMS, is that many digital health solutions are being offered directly to patients, with little-to-no provider visibility into how and when these tools are used. Providers must be able to capture the data that digital health technologies and instruments are collecting in patient medical records to ensure coordinated care. If patients are making health decisions with information from a third-party application or digital health service that is not visible to their healthcare team, complications and adverse outcomes may arise. CMS and ASTP/ONC can act decisively to make healthcare providers the central integration hub for digital health technologies, which can in turn help bring down costs and improve outcomes. CMS should carefully consider what digital health technologies can be managed and distributed through the provider organization to add additional value to covered patient care, then align incentives and reimbursement policy appropriately. Through this approach, CMS can bring digital health technologies into a whole health model, where remote patient monitoring, digital appointment management and other digital health technologies work in concert towards whole-person care.

Conflicting Market Incentives

Market incentives – and a false sense of data ownership – have allowed companies to profit extensively by limiting access to patient data. Existing incentives for data sharing and disincentives against information blocking have done little to change this paradigm.

CMS and ASTP/ONC must comprehensively address information blocking before healthcare data becomes further trapped in a limited number of competing vendor ecosystems. Information blocking is one of the biggest barriers to unlocking value from digital health technologies, as well as the bedrock of anticompetitive practices that discourage innovation and competition. The lack of enforcement of information blocking provisions has encouraged organizations to invest in lawyers to argue exceptions and technicalities rather than invest in good-faith interoperability efforts. If large technology vendors like EHR companies are regularly allowed to force each individual digital health vendor to jump through a myriad of technical and procedural hoops to connect an application, it makes competition and innovation cost-prohibitive, driving down the value of new digital health tools. In the current ecosystem, a new technology from a third-party developer must navigate constantly shifting regulatory policies as well as unique business policies from large technology vendors regarding patient access, obscure warning labels discouraging the use of third-party tools, or poorly functioning API plug-ins and intentional technical limitations.

By contrast, EHR vendors can develop their own digital health solutions for a fraction of the cost and effort described above. These products can be enabled for patients or providers by default by the vendor, sidestepping the artificial roadblocks that third-party developers face. The appeal of using a solution already integrated with the EHR largely comes from this ease of access – EHR-developed applications do not incur the costs that they impose on third parties, which are then often passed on to end-users. EHR vendors are also incentivized to develop their own patient record-sharing platforms, which only connect hospitals and health systems that operate on that vendor's product suite. This helps large EHRs keep patient records within their own system, deriving further commercial value from large quantities of walled-off patient data. These dynamics limit quality and competition, nudging providers towards "good enough" solutions.

To reverse this trend, efforts must be taken to ensure information blocking regulations more equitably disincentivize information blocking by all actors across healthcare, not just providers. When ASTP/ONC first pursued anti-information blocking measures in the Cures Act final rule, national survey data from 2021 found that roughly 36 percent of hospitals considered other healthcare providers to be engaging in information blocking, the highest among any type of healthcare actor. A year later, the same data showed only 12 percent of hospitals perceived providers as engaging in possible information blocking. Despite the data trending in a positive direction, the disincentives for information blocking included in 2024's HTI-1 final rule doubled down on hospitals, imposing harsh penalties – such as potentially not being considered a meaningful user under Medicare's Merit-based Incentive Program or Shared Savings Program – that would stifle the movement to value and place an outsized burden on providers.

In past rulemaking, ASTP/ONC built out a definition of information blocking via an ever-expanding set of exceptions. Despite ASTP/ONC's recent efforts to provide concrete examples of certain behaviors that could be considered information blocking, which may help providers remediate confusion and remain compliant, this model is rife with grey-area abuses that suppress competition and hold back interoperability.

Premier is concerned by the effect of the exemption-based structure and enforcement on competition in healthcare. As certain exceptions are extended to include all protected entities, it creates more situations where healthcare entities with different capabilities can leverage exceptions to protect and monetize data, circumventing the intent of information blocking regulations. Information blocking exceptions as written have created a market structure where some actors are able to use the exceptions process to set up barriers to data sharing and access. This creates a secondary market for health data completely independent of providers, one where EHR vendors, health IT solutions and data companies buy and sell patient data with no direct connections to quality or cost of care. Rather than unlock the flow of data between healthcare organizations, information blocking regulations have had the opposite effect.

Hospitals rely on data tools and analysis to improve patient outcomes, unlock cost savings, analyze population health and participate in innovative value-based care models. Interoperability regulations were intended to unlock data-driven healthcare, increasing market competition and ending unfair practices. However, unbalanced penalties allow large and litigious market leaders to build exemption-fueled moats around the data that hospitals are compelled to share. Each information blocking exception has become a barrier that hospitals and digital health companies must overcome to compete in the market. As a result, hospitals must buy back their own patient data in different formats, incentivizing certain actors to block their competitors from accessing data and competing on the merits of services offered. This drives up prices and allows market leaders to prevent new entrants from competing against their own data and analytics tools. As AI becomes more engrained into healthcare functions, these trends will amplify, leading to vertically integrated data solutions that weaponize information blocking exceptions to preclude new market entrants.

IV. HARMONIZING DIGITAL INFRASTRUCTURE: HOW CMS CAN SHAPE COMPETING INTEROPERABILITY FUTURES

Over the past two decades, CMS and ASTP/ONC have explored numerous different interoperability infrastructures, each of which has faced hurdles to successful implementation and some of which have come into direct competition.

Premier believes that CMS can leverage a range of authorities and policy levers to bring each of these programs into alignment with agency priorities and create a strong, interoperable foundation for the future of healthcare. ***Premier does not view these interoperability recommendations as mutually exclusive or interdependent; rather, Premier hopes to outline the current interoperability landscape that patients and providers must navigate, as well as a menu of policy levers CMS can use to improve practical interoperability.***

1. Meaningful Use and the Promoting Interoperability Program

CMS can modify meaningful use criteria to better reflect the actual uses that drive improved patient outcomes, correcting a growing disconnect between CMS program technology use measures and real-world problem sets. This disconnect is particularly evident in the gap between the intention and reality of CMS's Promoting Interoperability Program. The Promoting Interoperability Program and the definition of meaningful use were designed to incentivize and drive the adoption of EHRs. Initially, the decision to require providers to affirm their broad use of EHRs served its purpose, but these measures have become divorced from truly meaningful use of modern technologies. Today, in a world of data-driven care, meaningful use should be tied to the upper bound of data exchange and use, not the minimum capabilities of a decade prior.

EHRs are primarily vehicles for patient data, a clear purpose that fills a very specific role in the healthcare data ecosystem. Providers derive utility from a single patient record in their health system, and EHRs have, over time, driven efficiency in individual patient management and record-sharing. However, providers and public health officials also care about population health, data analytics and longitudinal patient outcomes. To truly unleash the potential of population health data analytics at scale, the Promoting Interoperability Program needs to evolve to shepherd this future.

EHRs are built for the transactional care of a patient during provider interactions, whether in a hospital or community-based setting. Providers and health systems need to share data with many other organizations for continual process improvement, care coordination, quality improvement and benchmarking. This requires levels of data enhancement that cannot be easily facilitated through transactional records alone. The data in the transactional systems must undergo stringent validation to ensure that the data quality is sufficient to compare across health systems and instill confidence in decision-making about changes to important processes such as physician workflows and supply utilization.

The datasets used for this type of work are large and multi-faceted, often including historical data. Health systems typically commit significant financial and workforce resources to data extraction and management, or, more frequently, hire a third-party vendor to write queries to extract data in a complete, usable format. The Promoting Interoperability Program does not require EHR vendors to make the full EHR data easily accessible in an all-encompassing and usable format. Furthermore, information blocking requirements – and the booming market for bulk health data – incentivize EHR vendors to further block off this data through legal and technical moats.

The transactional system of record captures, stores and has access to vast information that is not easily available and shared within the current standards. Within FHIR there are 160 base resources, and in the older HL-7 2.7 183 segments. Even if an average base resource or segment has an average of 20 elements, this leaves access to approximately 3,000 to 5,000 unique elements. Transactional EHRs store thousands of unique tables, 10 to 100 times the information available in the standards**.** Thus, critical operational data is trapped in the transactional system of record, not shared, and virtually inaccessible to the latest AI and other analytics tools to improve the cost and quality of care.

The quality of healthcare could be accurately assessed, costs reduced, and patient experience and outcomes improved if CMS considers updates to make a fuller data set easily accessible. Modifying the certification standards for health IT, meaningful use criteria and overall incentives to include data usability and availability for quality, benchmarking, and process improvement activities unlock the potential of healthcare data. It would be unreasonable and anticompetitive to simply require EHR vendors to provide these services, but by allowing health systems and third-party vendors to access data, CMS could make it easier for hospitals to modernize their digital infrastructure and transform patient care and payment.

2. FHIR-Based Digital Quality Reporting

Hospitals have a variety of basic data needs and priorities that are no longer fully aligned with the metrics incentivized in existing meaningful use and interoperability regulations. The development of interoperability, from the adoption of EHRs to the development of FHIR APIs, has transformed healthcare. However, it has become more complicated to maintain and update the existing web of regulatory definitions, enforcement authorities and agency organization that has developed over the past two decades. It is necessary to reduce unnecessary regulatory burden and remove conflicting incentives to enable data-driven healthcare processes beyond those envisioned when foundational interoperability rulemaking occurred.

Data Standardization for Quality Measurement and Reporting

Providers and other healthcare stakeholders have expressed concerns about the strain that Bulk FHIR APIs place on healthcare systems – sharing too much data through these APIs can slow down systems used for patient care and take up valuable system resources. Even more broadly, CMS should consider how the system of FHIR-based reporting that this initiative requires will shape the future of healthcare interoperability and data sharing. As Premier has commented previously, the interoperability and data sharing infrastructure resulting from the past decade of rulemaking has resulted in misaligned incentives that impede, rather than facilitate, the types of data uses necessary for modern healthcare technological advances, including AI.

Health IT vendors have historically created point solutions or tools that fit a specific compliance or regulatory requirement. It is reasonable to expect vendors to produce tools that will serve the specific quality reporting and interoperability requirements contained in ONC/ASTP rulemaking, meaningful use requirements, and eCQM reporting programs. ***CMS must ensure that any requirements or interoperability mechanisms developed to promote eCQM reporting do not complicate bulk data sharing for other crucial purposes, such as patient care coordination, analytics, AI or benchmarking to understand care or cost opportunities.***

For example, clinical trials, research, and even HHS's own stated vision for data-driven care require the ability for hospitals to share large historical datasets. For the average study, researchers must collect, share and query historical data spanning 2-3 years – even extending back 10-15 years for studies or trials with a narrow focus requiring longitudinal data. FHIR is a real-time data sharing framework that is unable to meet these demands leaving health systems to spend precious resource time writing queries because some EHR vendors block direct access to EHR source systems for third-party vendors and research programs to extract data themselves.

Current Bulk FHIR APIs do not have the ability to transmit the quantity of data, historical data sets, or types of data that researchers need. Furthermore, FHIR is primarily useful for transmitting structured data – but the future of data-driven healthcare lies in unstructured data such as bulk narrative notes or ambient listen voice, or even images that provides insights into the patient care journey and condition beyond the classes and elements contained in USCDI.

HHS must ensure that it does not force the healthcare sector to commit extensive resources to developing the technical capabilities for FHIR-based eCQMs at the expense of other interoperability needs. A future where compliance and reporting drive EHR vendors, payers, and providers to build different data sharing mechanisms – each costly and technically demanding – to fall short of the great promise in today's data-rich healthcare ecosystem where AI advances in days, not months or years. This is the environment facing American healthcare today, and HHS should make every effort to simplify the regulatory and technological barriers to true interoperability in future iterations and programs.

Timeline Under Consideration for FHIR-Based eCQM Reporting

The two-year timeline proposed by CMS in the recent FY26 IPPS Proposed Rule is untenable. Premier understands from conversations and business relationships with hospital members – work that includes hybrid dataset activities over the past decade of data standard evolution – that hospitals will need a

minimum of four years to fully and successfully transition to FHIR-based eCQM reporting. This is not to say that no hospital will be able to report eCQMs via FHIR within that timeframe, but rather, that a meaningful majority of hospitals will require roughly four years to effectively implement, troubleshoot, and optimize FHIR-based eCQM reporting. This extended timeline is due in part, but not exclusively, to the following complications:

Troubleshooting and compliance: The transition to a new reporting mechanism and data file type will require troubleshooting, both within health systems and within CMS. Hospitals rely on EHRs to map data correctly, and integrating these technologies can prove challenging – particularly if hospitals are not able to access and monitor these quality measures prior to reporting.

Bulk FHIR complications: Large health systems and hospitals must rely on Bulk FHIR APIs to exchange or share data. This process can be slow and cumbersome as Premier has heard from members that Bulk FHIR APIs can drain system resources and slow down other processes. The Bulk FHIR reporting process that would be necessary for large health systems or population-level data is a separate challenge from the standard FHIR eCQM reporting process and requires additional time and investment before it can be put into practice.

Resource gaps between EHR/Health IT vendors and hospitals: Much of the interoperability rulemaking that came from the 21st Century Cures Act focused on developing FHIR capabilities through EHR vendors, often via the Health IT Certification Program. However, even if EHR vendors can create and transmit eCQM reports to CMS via FHIR, hospital staff must be able to understand the process of data cleaning and reporting using their EHR. Insufficient resources have been dedicated to preparing hospitals and providers for this transition, including the support and education necessary for compliance with new requirements. Furthermore, given CMS's stated goal of eventually integrating non-EHR data sources into quality reporting, it is crucial to invest in building reservoirs of technical knowledge at the provider level – not just among health IT vendors.

Data capability differences across hospitals and health systems: Across several federal healthcare initiatives, it has become apparent that there is a gap in capabilities and resources between different hospitals in America. To truly achieve FHIR-based quality reporting nationally, policies must account for these differences. Lack of technical or financial resources in some hospitals, specifically rural and lower income areas, can raise issues of data consistency and validity. CMS must invest in consistent data capture and make it easy for hospitals to test the validity of data captured after the fact – and before it is reported to CMS. Vendors who work across the nation can write programs that are easily reusable from one health system to the next to eliminate each individual health system's need to write these programs and minimize cost across all health systems. Yet some EHR vendors block these entities from directly accessing source systems as a business strategy. Health systems then are forced to either hire additional in-house resources or contract out to EHR source system approved vendors to do this work, adding unnecessary IT costs to the overall health care delivery system.

One potential resource that CMS could provide to help accelerate the transition to fully FHIR-based eCQM reporting would be an open-source QRDA file to FHIR conversion tool. A confidential sandbox would allow hospitals to use this tool to troubleshoot and test FHIR-based reporting for certain data classes prior to full implementation. This tool and testing mechanism should be made available to both vendors and hospitals so that data analytics and performance improvement partners can also make the transition in a timely manner, supporting and accelerating the transition to FHIR-based reporting that CMS intends to achieve.

The challenges – and opportunities – outlined in Premier's comments are specific to the interoperability and quality reporting landscape facing individual providers in the IPPS program. ***For Accountable Care Organizations (ACOs) and Medicare Shared Savings Program (MSSP) participants, the additional barriers posed by collecting and reporting data from multiple unique EHRs with separate and unequal data capabilities and accessibility further complicate the transition to FHIR.*** Premier urges CMS to ensure that ALL mandatory tech-enabling and reporting programs move healthcare closer to a truly interoperable future, not a system where a different mechanism is required to share data for each use case.

3. Information Blocking

Information blocking disincentives should support the goal of longitudinal patient records, enabling value-based care and other quality-based metrics. ***Premier believes that information blocking incentives and disincentives should focus on creating an environment where data, and the insights that analytics provide, can be harnessed to produce better patient outcomes. Premier strongly urges CMS and ASTP/ONC to examine ways to equalize information blocking penalties and fairly redistribute the responsibility for ensuring free data sharing and interoperability between providers and other healthcare actors.*** HTI regulations have failed to prevent retention and siloing of large volumes of data, to the detriment of patient outcomes.

Although unintended and unanticipated at the time of promulgation, HTI rulemaking stemming from the Cures Act has created perverse incentives in the healthcare market. ***CMS and ASTP/ONC should investigate unanticipated effects on competition in the health data market of interoperability requirements and information blocking regulations. Such investigation would help illustrate the harms patients and providers experience from reduced competition in the provision of data-driven technologies in healthcare.***

The use of existing exceptions-based information blocking creates a system of uncertainty, where every potential case of information blocking must be evaluated individually against an ever-expanding list of broad and ambiguous defenses. This makes it challenging to investigate and prosecute information blocking because each complaint must be evaluated on a case-by-case basis, making clarification more likely than a penalty and further allowing offenders to drag their feet rather than provide interoperability.

CMS and ASTP/ONC must also make it easier for digital health developers to submit information blocking complaints, and to turn those complaints into meaningful enforcement action. This letter previously described some of the barriers that EHR vendors impose on third-party service providers and health IT vendors, limiting access to patient data and unfairly pushing patients and providers to use EHR-developed tools over competitors. When EHR vendors impose these additional technical barriers or engage in anti-competitive practices against third-party solutions, it may rise to the level of information blocking. By implementing a meaningful process for investigating, adjudicating, and correcting information blocking reports, CMS can ensure that innovation wins out, giving providers choice in the technologies they leverage to drive better outcomes and lower costs for patients.

Premier strongly urges CMS to begin information blocking enforcement in earnest. While the current exceptions-based information blocking regulations remain in place, the potential penalties – and CMS' willingness to impose them – must not be in doubt. Too many gray-area practices are considered a safe bet by EHR vendors, largely because these actors do not fear the potential penalties and have the legal resources to claim good-faith effort.

4. Certified Electronic Health Record Technology (CEHRT)

The CEHRT program represents the best tool to enforce interoperability requirements on EHR vendors, realigning incentives to make interoperability table stakes for health IT. CMS and ASTP/ONC should evaluate whether an ecosystem in which a small number of EHR vendors serve as default "operating systems" for providers is aligned with the Administration's policy goals, then consider using all available levers – including CEHRT certification – to correct any misalignment.

Healthcare providers have seen a significant change in the role of EHR vendors over the past decade. As healthcare's data engines, EHR vendors are empowered to produce software solutions for every healthcare function. EHR vendors also leverage their role storing a health system's data to require digital health developers to pay fees for each bidirectional instance that they set up for a tool, often reaching $25,000 to $50,000 per instance. This fee then gets passed on to end users of these digital health solutions. This ultimately results in hospitals paying EHR vendors once to collect data, once to store it and once more to

use it. This paradigm makes digital health technologies and services more expensive, which makes healthcare more expensive.

As noted above, beyond a limited subset of elements, standardization is missing from healthcare data. ***Premier encourages CMS and ASTP/ONC to leverage CEHRT criteria to require EHR vendors to report healthcare data – including quality measures, ADT and ADT notification, lab, and clinical data – using standardized data elements.*** This would simplify hospital quality and performance improvement efforts, reducing patient costs and driving better outcomes.

Finally, HHS can use CEHRT to raise revenue to fund information blocking enforcement and interoperability efforts. ***By imposing a user fee for the CEHRT program proportional to EHR vendor revenue, HHS could ensure that necessary information blocking complaint investigation and enforcement are not a drain on agency resources***. User fees will also provide CMS and ASTP/ONC with resources to promote interoperability pilot programs or new initiatives in the future. Revenue-proportional user fees will also ensure that the fees do not become a barrier to entry for new EHR vendors.

5. Blue Button 2.0

At the foundation of tech-enabled healthcare is data-sharing infrastructure. Throughout this letter, Premier outlines several different CMS policy authorities that could achieve functional interoperability and improve the ability to build longitudinal patient records across various provider organizations. Although information blocking and EHR-based interoperability are at the crux of today's interoperability debate, CMS could also consider building out existing infrastructure to better support the Blue Button model of patient-directed data sharing.

When the original Blue Button was developed, it allowed veterans and servicemembers to download their health records from the TriCare Online portal or Veterans Affairs' EHR. These individuals could then take those records to other providers. HHS soon adopted its own Blue Button for Medicare beneficiaries. In 2018, HHS announced Blue Button 2.0, similar in intent to the original Blue Button but functionally enabled by APIs. However, Blue Button 2.0 has been significantly hindered by limited uptake among major healthcare IT vendors.

To broadly expand the usefulness of the Blue Button 2.0 initiative in the spirit of the original Blue Button, CMS and ASTP/ONC should leverage updates to the Certified EHR Technology criteria to mandate that EHR vendors incorporate a Blue Button 2.0 API that allows patients to view, download or transmit their full electronic health record with a single click. The capability is well within current technical feasibility - allowing patients to access their records directly from EHR vendors rather than through providers is the next step to creating functional digital healthcare infrastructure. This mechanism would also allow patients to direct the sharing of their own health records with the provider of their choice – rather than continuing to promote a technology ecosystem where everyone but the patient has a say in data sharing and everyone but the provider derives value from building walls around data they are entrusted with. ***CMS and ASTP/ONC must ensure that EHR vendors are not the sole arbiters of data access. Mandating a patient-directed data sharing mechanism in CEHRT criteria could reconfigure how America thinks about ownership and sharing of patient data, restoring privacy and consent while building longitudinal records by default.***

6. Medicare Advantage Program

<u>Data Standardization</u>

Payers hold a wealth of healthcare data, including elements necessary to construct longitudinal patient records. However, payers are not currently incentivized to share this data with providers or patients outside of a limited number of alternative payment models. CMS has developed Blue Button 2.0, a claims data API that allows Medicare beneficiaries to access and download their claims data. However, commercial payers are not incentivized to offer similar transparent and usable data. CMS should encourage payers to report quality measures in a standardized format. Significant time and investment have been spent on

standardized quality measure specifications across payers – similar requirements should be added to the output and sharing of these measures as standardized data elements.

Premier recommends that CMS promote standardized data sharing by payers through the Medicare Advantage (MA) program. CMS should require MA plans to utilize a standardized quality measure in reporting to providers as part of its regulatory authority. This could be achieved by imposing a direct regulatory requirement for payers contracting with CMS as part of the MA program. However, if CMS believes that imposing this requirement would exceed its regulatory authority, it could alternatively request that CMS-regulated health plans report quality data to CMS in a standardized format. CMS could then publish this data and make it available to providers in real-time via FHIR API. CMS can also use its authority to require MA payers to implement a Blue Button 2.0 API that is accessible to patients and providers.

Leveraging Electronic Prior Authorization

A critical component of the healthcare technology ecosystem in the MA program is the exchange of data between providers and payers for prior authorization and claims payment. As policymakers consider requirements for MA plans to utilize electronic prior authorization, ***Premier encourages CMS to go a step further and stipulate that claims approved under an electronic prior authorization may not be artificially delayed or denied***. In essence, electronic prior authorization should serve as an agreement between the payer and provider to perform the requested service *and* remit timely payment for it. It is critical that federal regulators leverage the technology pathways that already exist to streamline prior authorization as soon as possible, rather than waiting years for health plans to catch up.

Improving Data Validation Audits

Premier supports the Administration's efforts to improve documentation and coding accuracy in the Medicare Advantage program. However, restricting this effort to the existing Risk Adjustment Data Validation (RADV) program would involve significant investment in expanding the federal coding workforce and acquiring and deploying technology. ***Premier urges CMS to leverage the private sector's investments in technology-enabled self-audits to improve compliance while limiting administrative burden and CMS operational costs.***

The current regulatory framework for RADV involves auditing a sample of records for each health plan and extrapolating plan compliance based on a statistical analysis of the sample. In its recent announcement, CMS noted that the agency's revised plan for RADV audits would involve increasing its team of medical coders from 40 to approximately 2,000 and deploying new technology to flag unsupported diagnoses for documentation review. While this expanded effort may lead to increased recoupments of overpayments for previously unsupported diagnoses for risk adjustment, audit outcomes are ultimately subject to a brittle and burdensome chart submission process. There is a more compelling technology solution: CMS can enlist the technology that payers and providers are already implementing for self-audits to conduct compliance audits on the agency's behalf. ***In parallel to increasing RADV audits, Premier recommends that CMS test the use of technology to create a real-time documentation validation.***

A large percentage of payers and providers currently utilize a documentation and coding self-auditing solution. Wider adoption of coding accuracy software yields a positive outcome for payers, providers and CMS, ending the pay-and-chase model of Medicare program integrity monitoring and offering greater confidence to the private sector around compliance rates. Leveraging its existing authorities, CMS could pilot a program in which MA plans that are using a CMS-approved self-auditing solution could substitute self-funded, tech-powered audits, noted on encounter data records or via a direct feed to CMS, for RADV audits. To spur uptake, ***Premier recommends that CMS require MA plans to pay a user fee, sufficient to ensure that plans are not buying their way out of real-time audits, if the plan prefers for CMS to conduct a post-payment RADV audit on the plan's behalf.***

7. CMS Quality Programs

Through its quality programs, CMS has the authority to set standards for reliable, safe, and high-quality patient care, as well as the ability to implement new measures that take advantage of the wealth of data available in the healthcare ecosystem to deliver better care at lower costs for patients.

One of the most straightforward steps CMS can take to improve the availability and usefulness of quality data is to reduce lags between quality data generation and reporting. Both claims and non-claims quality data sources have lags of up to or longer than 12 months between the provision of care and data on the quality of care – even for simple measures such as Patient Safety Indicators or infection counts. While these lags don't prevent benchmarking, data that isn't available to providers until months later makes constant quality and performance improvement nearly impossible. ***Premier urges CMS to evaluate how data standardization and reporting requirements can leverage technological capabilities of EHRs and payer-provider data exchange to provide providers with even preliminary quality measures as close to real-time as possible.*** Researchers have already shown that large language models (LLMs) can accurately predict hospital quality measure performance. By leveraging these capabilities, CMS can ensure that health systems have real-time visibility into quality data, allowing providers to quickly address shortcomings and improve performance within the same reporting year.

Additionally, CMS has invested heavily in collecting and reporting quality metrics across health systems, but more robust attribution methods can help CMS and patients to understand quality of care attribution at the physician level. Premier researchers have developed a novel approach to determine physicians' relative degrees of responsibility for inpatient stays through physician-specific attribution ratios informed by existing patient characteristics and comorbidities. CMS should take measures to support the reporting of physician-level outcomes attribution to better inform patient choice and drive healthcare sector improvement.

Device Identification for Patient Safety

The Food and Drug Administration's (FDA) Unique Device Identification (UDI) system provides each medical device with a code corresponding to its make, model and other clinically relevant information, such as expiration date. The UDI system enhances recall resolution and enables more sophisticated postmarket surveillance. However, the full public health and patient safety benefits of this new device identification system are only possible through UDI capture and transmission throughout healthcare delivery, including in electronic health records, adverse event reports, supply chain systems and claims.

Unlike many other information sources, claims offer large, standardized data sets for analysis. They also support longitudinal analyses on patient outcomes across multiple providers. ***Premier urges CMS to require the inclusion of medical device identification information in the EDI 837 transaction electronic claim form.*** The addition of medical device identifiers to health insurance claims forms will help ensure more accurate adverse event reporting, enable improved evaluations of marketed devices, reduce medical errors through improved device identification, decrease healthcare supply chain costs, and facilitate more comprehensive recall resolution. Patients, clinicians, providers, policymakers, regulators and researchers will have better data to help ensure high quality care, prevent harm and reduce cost. The UDI system can only achieve its potential to improve patient care once it is included in health insurance claims forms. Claims provide longitudinal information on patient care delivery and outcomes across the continuum of care, sites and providers. The incorporation of device identifiers to claims will provide an efficient way to understand the long-term safety and performance of different devices. Particularly in an era of value-based care, healthcare providers need this information to make informed decisions about the products they use.

Premier shares the vision of enabling healthcare providers to track medical devices electronically within the supply chain and across the care continuum. By exercising its authority to require the inclusion of device identifiers in claims, CMS can help improve provider understanding of device effectiveness and ensure patient safety related to adverse events.

8. Trusted Exchange

CMS should fully participate in data exchange infrastructure. Should CMS choose to join a trusted exchange framework, CMS can use its authority to address information blocking concerns, ensure that participants cannot use the Framework to wall off health records from use for quality and process improvement by providers and their vendors, and facilitate requests for CMS and CMS-regulated health plan data.

There has been a great deal of debate – both legal and policy – about the ability of trusted exchange participants to accept or deny requests for access to patient data. It is important that patient data privacy is the expectation, and participation in a trusted exchange should not be abused; however, it should also not be up to private companies with a vested commercial interest in charging for access to patient data to determine which data requests to grant beyond confirming the request is consistent with the purposes of use. It should be up to CMS and ASTP/ONC, not individual companies, to regulate participation. CMS or ASTP/ONC should use existing authorities to adjudicate violations and make determinations about an organization's ongoing ability to participate in a trusted exchange.

By participating in a trusted exchange framework, CMS can also facilitate data standardization, collection and sharing. CMS programs have access to the data of Medicare beneficiaries, Medicare Advantage plans, state Medicaid programs and dually-eligible beneficiaries, and CMS can and should require these entities to report a full range of data in a standardized format, then facilitate the sharing of this data to patients and providers.

The value of CMS participation in trusted exchange is twofold. First and foremost, CMS can quickly and easily share data on Medicare and regulated health plan beneficiaries with patients and providers, offering additional visibility into a patient's care journey and preventing unnecessary complications. CMS may not derive much value from querying data from other exchange participants – formal data request and reporting authorities described elsewhere in this letter are more efficient for CMS data collection. Finally, through participation in a trusted exchange, CMS can provide a trusted, low- to no-cost onramp into the health technology ecosystem for patients and providers – reintroducing data accessibility and exchange to a system that has been undermined by the actors charging steep fees for exchange access.

V. AMERICA'S HEALTHCARE FUTURE: TECH INFRASTRUCTURE THAT WORKS FOR VALUE-BASED CARE (VBC)

The vision for value-based care is to improve the patient experience, improve population health, reduce costs and improve provider well-being. With the advent of AI, digital health apps for patients, and API-based interoperability, current technological advances are positioned to leapfrog each of these goals through their integration with value-based care. Combining team-based care with tools that enhance both care delivery and the patient experience will help keep patients healthy and reduce costs, while also addressing workforce challenges and clinician burnout. For example, a Premier member health system that also operates a REACH ACO successfully increased patient engagement through digital outreach technology that simplified scheduling and automated many outreach functions among high-risk populations. The tech-enabled solution decreased care gaps without adding additional workload to the patient's care team.

To achieve this level of seamless care on a broader scale across VBC models, it is essential that CMS understand challenges with prior attempts to incentivize the use of technology in VBC and leverage these learnings in the development of practical and sustainable policy solutions. ***Premier urges CMS to incorporate the following principles in the development of policy to support the increased adoption of technology in VBC models:***

- ***Understand that VBC models often operate within an ecosystem of providers with varying levels of technical sophistication and different EHR technologies that may not be interoperable;***
- ***Monitor for unintended consequences of new policies and make adjustments accordingly;***

- ***No unfunded mandates;***
- ***Clearly define the patient outcomes CMS wishes to achieve and permit VBC model participants the flexibility to innovate to meet those end goals; and***
- ***Leverage a carrot approach to financial incentives versus a stick approach;***
- ***Level the playing field so that all VBC participants have an equal opportunity to implement technology;***
- ***Proactively seek stakeholder feedback in the development of new payment models and refinement of existing VBC programs on the integration of novel technology.***

What Not to Do: Lessons Learned from MSSP Quality Reporting Changes

Since 2023, CMS has worked diligently to align quality measure sets across programs (e.g., Medicare, Medicaid, and Medicare Advantage) and increase electronic clinical quality measure (eCQM) with the goal of reducing administrative burden on providers and streamlining quality measurement processes. While CMS' vision to transition to fully digital quality measure (dQM) reporting is admirable, the unintended consequences of decisions made to execute that vision have increased administrative burden for ACOs and driven more independent practices away from Alternative Payment Model (APM) participation. If CMS wants to enhance the success of APMs and increase APM participation by independent practices through the promotion of tech-enabled solutions, then CMS should carefully evaluate the pathway towards that vision and the effects of its decisions given the current state of the technical capabilities of providers. Below we describe policies previously flagged by Premier that had the intention of reducing provider burden and increasing interoperability, but produced the opposite outcome.

Example 1: In the Biden administration, CMS finalized a policy to phase in the APM Performance Pathway (AAP) Plus,[1] which includes all-payer eCQMs, and sunset the Web Interface beginning in 2025. While the intention of this policy was to increase electronic reporting of quality measures, it ignored the reality of the current interoperability limitations, given how many ACOs are structured. ***ACOs experience considerable limitations due to the lack of interoperability among the many EHRs in their network and a huge burden associated with eCQM reporting.*** It is critically important to understand that ACOs vary widely in their electronic data extraction and aggregation capabilities**.** It is true that some ACOs have a single EHR system that permits eCQM submission for an entire organization, but more commonly ACOs are formed (as contemplated in statute and regulation[2]) by groups of providers and suppliers that have multiple different EHR instances, sometimes hundreds, each with varying levels of CEHRT standards. Combining data from multiple EHRs to produce a single result is not a capability that most ACOs have. Nor do all ACOs have the capability to submit deduplicated patient data as required by CMS.[3] Although CMS released an open source "dedupliFHIR" tool to assist ACOs in data aggregation and patient matching, it does not address all the technical challenges and concerns.

CMS should recognize that ACOs are fundamentally different from clinicians and groups whenever it develops new tech-related policy. ***The reporting requirements finalized in the CY 2025 Physician Fee Schedules (PFS) essentially align the MSSP quality standard with MIPS, which is a fundamentally flawed approach.*** ACOs reflect coordination of care across the continuum, as compared to MIPS, which reflect point-in-time encounters by individual clinicians and groups. ACOs are a network of aligned providers rather than a specific provider type. All-payer measurement is ideal for setting provider-specific measurement as it holds providers accountable for their entire patient population. ACOs are held accountable for cost for a defined patient population by partnering with providers to innovate and coordinate care. ACOs themselves do not

[1] Federal Register:: Medicare and Medicaid Programs; CY 2025 Payment Policies Under the Physician Fee Schedule and Other Changes to Part B Payment and Coverage Policies; Medicare Shared Savings Program Requirements; Medicare Prescription Drug Inflation Rebate Program; and Medicare Overpayments
[2] 42 CFR 425.102(a)
[3] Medicare Shared Savings Program - Reporting MIPS CQMs and eCQMs in the Alternative Payment Model Performance Pathway (APP): Guidance

directly provide care. Requiring ACOs to report all-payer data is comparable to requiring health plans to report on other payers' populations. Instead of building ACO quality reporting based on the structure used for individual clinicians, Premier strongly urges CMS to look to how quality reporting is conducted by health plans. Like health plans, CMS could adopt a similar system used for HEDIS reporting, which combines data from multiple sources, including EHRs, clinical registries or health information exchanges (HIEs), case management systems and claims data.

CMS should continually monitor the unintended consequences of its policies. Given the challenges associated with the new APP-based quality reporting requirements, some ACOs are considering narrowing their participants list, including removing specialists. This will ultimately hinder CMS' ability to align all Traditional Medicare beneficiaries with ACOs and increase specialist engagement. All-payer measurement could also significantly impact ACO performance on certain preventive care measures for which historically ACO specialists are not accountable. For example, most orthopedists or ophthalmologists do not routinely screen patients for depression. However, all-payer APP-based quality reporting could cause their patients to be included in the denominator of this measure and adversely affect the ACO's measure performance score. As a result, some ACOs are considering removing specialists from their ACO. As smaller practices consider the business case for significant investments into their IT capabilities, some are likely to determine that continuing to partner with an ACO is no longer feasible.

Example 2**:** In the prior Administration's CY 2024 PFS rulemaking, CMS aligned mCERHT requirements for MSSP with MIPS. Beginning in performance year (PY) 2025, MSSP ACOs must use MIPS Promoting Interoperability (PI) performance category measures and scoring, regardless of QP status. Although the intent of this policy was to streamline programs and promote the uptake of CEHRT within ACOs, some ACOs may be forced to remove independent practices from their network who lack CEHRT. Premier is concerned that this policy places excess burden on ACOs and clinicians, contradicts the statutory exemption from MIPS as a key incentive for clinicians to participate in Advanced APMs and fails to recognize the key distinctions between ACOs and individual clinicians or groups.

Rather than promoting interoperability, this policy places additional burden on ACOs (or individual clinicians). It is a cautionary tale for policy related to advancing technology adoptions. ACOs are already overburdened and face many challenges that impact their stability, including the expiration of Advanced APM Incentive Payments at the end of CY 2024. In addition, according to statute, QPs in an Advanced APM are exempt from MIPS, which is a valuable incentive for clinicians to join an Advanced APM. Removing this incentive and requiring these clinicians to meet MIPS Promoting Interoperability requirements is counter thetical moving more Medicare providers into APMs.

Incentives for Technology Adoption in VBC

For accountable care organizations, the primary incentive for encouraging the adoption of digital health management, care navigation, and other tech-enabled technologies is adequate financial incentives that can be re-invested into such tools. ***ACOs need upfront investment – whether in the form of prepaid Shared Savings, one-time investments, or enhanced interim capitated payments that are paid back over time – that is available to all ACOs, regardless of size, organizational structure, or "high revenue" status.*** There is seemingly a misperception that all health system-led and hospital-led ACO's, typically categorized as "high-revenue" ACOs, have access to readily available capital and IT infrastructure that is not otherwise available to provider-led ACOs. However, not all hospitals are the same as many continue to struggle with financial viability given ongoing reimbursement cuts and rising costs associated with inflation, workforce, and more resulting in an inability for many hospitals to invest in technology. Therefore, even health systems that qualify as "high-revenue" need access to capital and unlike some provider-led ACOs, they rarely have venture-backed conveners supplementing them with modern technology, data informatics, and AI capabilities.

Incentives for promoting the uptake of tech-enabled solutions cannot be unfunded mandates and should be "carrots" rather than "sticks." For example, providers could receive financial incentives when they demonstrate successful adoption of particular tech-enabled solutions or exemptions from relevant reporting requirements. What is not advisable is reducing payment to providers who fail to adopt tech-enabled solutions as that would create a vicious cycle and is a known criticism of the Promoting Interoperability program. All providers should be given equal opportunity and a glidepath for modernizing their IT capabilities. Moreover, there are often unintended consequences when requiring technology adoption – as seen in the case with promoting CERHT – that has the opposite effect of creating more administrative burden rather than operational efficiencies and interoperability.

In addition, it is critical that CMS clearly define the patient outcomes it wishes to achieve and permit VBC model participants the opportunity to innovate to meet those end goals. A one-size fits all approach should not be pursued as model participants vary in their size, scale, and sophistication often warranting individualized approaches for the communities they serve. Instead, it is important for end goals to be clearly articulated so that participants can innovate and determine how to best meet those goals while optimizing existing resources and expertise.

As noted above, the purpose for expanding the integration of technological advances into VBC is to improve the patient experience, the provider experience and population health, while also reducing costs. Presently, a number of headwinds challenge the viability of VBC within Medicare if CMS does not take corrective action. Since passage of the Medicare Access and CHIP Reauthorization Act of 2015 (MACRA), unrelated actions by both Congress and CMS have eroded a hierarchy of incentives that were made available to Medicare Alternative Payment Model participants.

- Level 1 Incentive: A level one incentive was the Advanced APM bonus payments to Qualifying APM Participants (QP) – providers who take on meaningful downside risk – which expired at the end of calendar year (CY) 2024. A distinctive feature of the Advanced APM bonus is that it was agnostic to the financial methodology of the APM and that it was guaranteed. Meaning, QPs received the bonus regardless of how well they did or did not perform in their APM. Moving forward, payment updates to QPs for physician services under the Professional Fee Schedules (PFS) would increase by the APM conversion factor of 0.75 percent or by 0.25 percent for non-QPs.
- Level 2 Incentive: Level two incentives are the payment incentives intrinsic to the design of the APM. These intra-payment model incentives are highly dependent on financial methodologies and not guaranteed as it depends on a provider's performance.
- Level 3 Incentive: MACRA also established a third incentive in APMs with significant downside financial risk: exclusion from MIPS reporting, which many providers find burdensome.

A combination of Congressional and CMS actions has eliminated the first and third incentives, leaving only the second, which is not guaranteed. If Congress lets the Advanced APM incentive expire and – as proposed in the Budget Reconciliation Bill H.R. 1 passed by the House – enacts legislation eliminating the APM conversion factor scheduled to begin 2026, Congress will remove a critical incentive for providers to participate in APMs with downside risk. CMS further eroded the non-financial incentives for Advanced APM participation by 1) increasing reporting burden by requiring MSSP participants to report on MIPS Promoting Interoperability measures in the CY 2024 PFS final rule, regardless of QP status and 2) sunsetting the Web Interface in 2025 in favor of transitioning to MIPS eCQMs and requiring all-payer reporting. For reasons discussed below, these decisions eliminate any relief from reporting burden.

With this context in mind, CMS has a great opportunity to re-evaluate the kinds of incentives providers need to voluntarily assume downside risk. ***CMS should create a holistic framework of both financial and non-financial incentives to promote both VBC participation, which in turn results in greater adoption of tech-enabled solutions.***

Additional Opportunities for Promoting Adoption of Tech-Enabled Healthcare

AI and care delivery tools have the power to supercharge the effectiveness of VBC delivery models in reaching patients and keeping them healthy. Premier offers the following recommendations for further driving the adoption of technology in VBC models.

***Re-consider policies that require use of CEHRT within APMs*.** Increasingly, it is redundant to require PI measures for APMs when it is a necessary condition to operate CEHRTs before considering participating in APMs. For APMs that include multiple organization types coordinating care under a single entity, there should be flexibility for those organizations to work out their data and technology needs according to their own timelines. A prime example of this are post-acute care providers, many of whom would not qualify for inclusion in an ACO's participant network if they lack interoperable EHRs. ACOs do this on their own without any regulatory mandates because it makes business sense.

For many years, HHS has focused on policies that encourage and support the adoption of interoperable health information technology. While these efforts have included significant financial incentives for many providers, post-acute care providers such as skilled-nursing facilities (SNFs) and long-term care (LTC) providers have not been eligible for these incentives. ***Premier urges CMS to focus on advancing interoperability for all providers along the continuum of care, including SNFs.*** CMS' pilot testing of proposed SNF measures confirmed that the most common mode of information transmission to the patient and to the provider was paper-based. This long-standing reliance on paper-based transmission of information presents a significant barrier for post-acute care (PAC) providers to implement EHR systems and participate in value-based care arrangements. Additional barriers for PAC providers to adopt EHR systems include a lack of financial incentives under the Health Information Technology for Economic and Clinical Health (HITECH) ACT. As a result, many SNFs and other PAC providers are not using EHRs or are using EHRs that are not designed for interoperability. Providing financial incentives for CEHRT adoption by PAC providers is critical for their participation in APMs.

In addition to PAC providers, Premier members operating ACOs have shared concerns about the lack of data sharing between healthcare providers and retail pharmacy. Retail pharmacies are a segment of the healthcare industry whose technological capabilities are not advanced or fully interoperable. Moreover, because retail pharmacies do not typically participate in ACOs and APMs, they may ignore requests for data from ACOs. When they are responsive to data requests, the data they share may be in the format of PDFs and not interoperable. ***Therefore, CMS should also include retail pharmacies and long-term care pharmacies when developing incentives for adopting tech-enabled solutions*.**

Premier has previously submitted comments highlighting the burden on ACOs to notify beneficiaries of their ACO-alignment. It has been challenging for ACOs to design processes to ensure compliance with the timing of the notification, which requires the follow-up communication to occur at the earlier of the beneficiary's next primary care visit or 180 days after the initial communication. Since ACOs often do not have real-time insight into when a primary care visit occurs, some ACOs are considering sending the follow-up notification soon after the initial notification or requiring the follow-up communication to occur at every primary care visit to ensure compliance with the requirement. This introduces additional burden on ACO professionals, as well as increases potential confusion for beneficiaries. ***Consequently, the notification to beneficiaries of ACO-alignment could be an opportunity for CMS to work with stakeholders on developing an automated, electronic notification process that is clearly understood by beneficiaries and adds overall value rather than serving as a "check the box" activity.***

As previously shared, ***Premier strongly supports the extension of the telehealth waivers that were used during the Public Health Emergency (PHE) as a critical component of tech-enabled healthcare***. Specific telehealth waivers could include 1) expanding the providers eligible to for telehealth; 2) allowing use of audio-only telehealth for certain services; and 3) making permanent inter-state waivers for telehealth appointments to allow providers to deliver services across state lines without the need for multiple state-specific licenses. While advocates for telehealth waivers are seeking Congressional action, CMS can also enact change by allowing CPT codes for telehealth services to be covered under the regular Medicare

benefit, not telehealth benefit. If CMS makes these codes billable, then Congressional action may not be necessary. These enhanced flexibilities would further increase VBC penetration to regions, such as rural communities, where there are significant provider shortages, and have lasting effects on both Medicare and healthcare delivery more broadly.

Finally, with proper privacy safeguards in place, digital identity credentials could be very useful for APM participants and especially ACOs. Similar to the Orange Banner in Epic, ACOs would have line of sight into when an aligned beneficiary receives care outside their network. This data would support ACOs in better managing the cost of care for a beneficiary, identify emergent needs, and coordinate with independent practices. Moreover, it would be data that is more timely than a claims feed from Medicare, which is often lagged. Patients would be more visible to ACOs and receive better care coordination.

Premier applauds CMS for seeking public comments on promoting technology adoption in VBC arrangements. ***As a practice, CMS should always proactively seek stakeholder feedback on the integration of novel technology when developing new payment models via the CMS Innovation Center or refinement of existing programs such as MSSP.***

VI. CONCLUSION

In closing, Premier appreciates the opportunity to provide feedback on opportunities for creating a stronger, more resilient, more functional digital health ecosystem. Premier looks forward to continuing to work with CMS and ASTP/ONC on implementing these recommendations to unleash innovation and improve efficiency in healthcare delivery.

If you have any questions regarding our comments, or if Premier can serve as a resource on these issues, please do not hesitate to contact me at soumi_saha@premierinc.com or 732-266-5472.

Sincerely,

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